

Recurrent Hypoglycemia After Increased Activity

Andrew · Type 1 · Omnipod + Loop (closed-loop) · Dexcom G7 · Review period: Jun 24 - Aug 23, 2026

Generated from Nightscout CGM & pump data by a family member. Observational — for clinical review, not a diagnosis.

Summary

Marching band began ~Aug 10, a sustained increase in daily activity. Since then **time below 54 mg/dL has risen to 2-3× the recommended <1% target**, concentrated in the evening (practice window) and in the following overnight/early-morning hours — a pattern consistent with delayed post-exercise insulin sensitivity. **Bolus insulin has already self-corrected down ~22%, but the basal schedule has not changed** and still reflects a sedentary summer at home. He moved into a college dorm Aug 21 and is now managing lows unsupervised (alarms are heard, fast carbs and a roommate are in place; all recorded episodes were self-treated and resolved).

1 · TIME BELOW RANGE, BY WEEK

WEEK	MEAN	TIR 70-180	% <70	% <54 (goal <1%)
July (reference, pre-band)	110	93%	5.1%	0.8%
Aug 3 - 9	113	89%	7.5%	0.8%
Aug 10 - 16	BAND STARTS115	88%	8.6%	3.1%
Aug 17 - 23	DORM AUG 21110	90%	7.4%	1.8%

Worst individual days: Aug 12 — 14.2% of the day <54; Aug 22 — nadir 40 mg/dL; Aug 23 — nadir 50 mg/dL. Overall glucose and A1c-equivalent remain good (mean ~110, GMI ~5.9%); the problem is hypoglycemia exposure, not average control.

2 · WHERE THE LOWS MOVED (BEFORE VS AFTER AUG 10)

WINDOW	%<70	%<54
Evening 17-21 practice	5.9 → 14.3	1.1 → 4.0
Overnight 00-06	8.7 → 7.8	1.1 → 2.9
Morning 06-12	3.2 → 6.3	0.5 → 2.7
Afternoon 12-17	7.9 → 6.9	0.9 → 1.7
Late 21-24	7.9 → 6.0	1.0 → 0.4

Evening is the epicentre; the overnight and next-morning severe lows roughly tripled, consistent with the 12-24h tail of post-exercise sensitivity rather than anything occurring at that hour.

3 · BY DAY OF WEEK (SINCE AUG 10)

DAY	MEAN	%<70	%<54
Wednesday	103	18.1	8.2
Friday	115	9.4	3.7
Sunday	115	3.7	2.0
Saturday	108	5.2	1.9
Monday	108	9.2	0.3
Tuesday	122	5.6	0.7
Thursday	116	4.2	0.5

Wednesday is a clear outlier — 8.2% of the day below 54. Worth confirming against the rehearsal schedule.

4 · INSULIN: BOLUS HAS FALLEN, BASAL HAS NOT

	BOLUS INSULIN	CARBS	SCHEDULED BASAL
Before Aug 10	43.3 U/day	178 g/day	38.1 U/day
After Aug 10	33.9 U/day (-22%)	150 g/day	38.1 U/day (unchanged)

Loop and the user have already compensated on the bolus side. The scheduled basal was last tuned Jun 20 for a low-activity summer at home and has not been revisited since activity increased — so the residual excess is concentrated in basal, which is precisely the insulin acting during and after practice.

5 · REPRESENTATIVE NIGHT — SAT AUG 22 (DORM)

00:30	88	→ "Eating Soon" override + 2.5 U
01:00	127	→ "Rage" override (1.2× insulin, target 80) + 2.85 U + 10 g
01:30	76	
02:00	50	
02:30	43	→ treated, 15 g + "Running Low" override
03:00	76	recovered

Post-midnight correction stacking on top of an already-heightened sensitivity state, followed by a crash ~90 minutes later. The overnight "🔥 Rage" preset (1.2× insulin, 80 target) was used 1-8 times per day across this period.

6 · CURRENT SETTINGS & QUESTIONS

Basal (38.05 U/day): 00:00 1.45 · 02:00 1.30 · 04:30 1.35 · 09:30 1.80 · 11:00 1.25 · 16:00 1.80 · 17:00 1.70 · 20:00 2.40 · 22:30 2.20
ISF 53 mg/dL/U · **Carb ratio** 6 g/U · **Target** 100 mg/dL · **DIA** 6 h · flat all day

- **Overall basal reduction?** Bolus has already dropped 22% while basal is unchanged from a pre-activity baseline.
- **An exercise override protocol around practice** — extending several hours past the session to cover the delayed overnight tail? Existing presets are 0.8× / target 120 and 0.7× / target 120.
- **Wednesday specifically** — 8.2% below 54 suggests that day needs its own plan.
- **Guidance on late-evening corrections**, given the post-midnight stacking pattern above.
- **ISF / carb ratio** — both flat all day and unchanged; should they move with the new activity level?